

31. DIAPHRAGM PRACTICE - 3 TECHNIQUES

DURATION : 16:09

STUDY SHEET

COURSE SUMMARY

This video presents two techniques for diaphragm release, essential for improving respiratory function and the general condition of patients, particularly children. The first technique focuses on the oesophageal and diaphragmatic junction, utilising spiralling movements and tissue listening to relieve pressure in the diaphragm. The detailed practical protocol includes steps for neck assessment, release, and mobilisation, aiming to restore freedom of movement. The second technique targets the omental bursa and oesophageal insertions, highlighting the importance of the interaction between the ribs and the diaphragm. These approaches, rooted in biodynamic practice, aim for the patient's overall well-being.

DIAPHRAGM PRACTICE - 3 TECHNIQUES

INTRODUCTION TO DIAPHRAGMATIC TECHNIQUES

We will explore two main techniques. The first is a **mechanical** approach to release the diaphragmatic space. The second is a **global** approach to the diaphragm, particularly effective in children and babies, to restore **optimal expansion**. This technique falls within a **biodynamic** practice.

TECHNIQUE 1: RELEASE OF THE OESOPHAGEAL AND DIAPHRAGMATIC JUNCTION

We will work specifically on the **junction** between the oesophagus and the diaphragm. This region is rich in **nerve networks**, particularly with the trunk of the **vagus nerve**. The objective is to gently release this area, which will have the effect of unloading the entire surrounding vagal area.

KEY ANATOMY

- We will observe the right and left diaphragmatic portions.
- The **pleura** is also present here.
- There is an anatomical remnant, formerly a vein, called the **vein of Treitz and Lehmer**.

- A small muscle inserts onto the diaphragm and peritoneum, forming the **peri-oesophageal** space. This space has a function of adaptability and rebalancing in relation to the diaphragmatic junction.
- This structure extends with the **muscle of Treitz**, which is the suspensory ligament of the duodenum.

THE BODY'S SPIRALLING MOVEMENT

The body is often organised in **spirals**, a powerful movement found everywhere. The embryo itself develops according to spiralling movements. We use spirals in energetic work to target precise points. Here, we will apply this principle of spiralling movement.

PRACTICAL PROTOCOL

1. **Location:** Position yourself at the junction, where you can access the highest part of the stomach.

2. Preliminary Neck Test:

- Turn the head to the left and feel the sensation.
- Return to the centre, then turn the head to the right.
- Return to the centre, then turn the head to the left again. Note if the rotation is limited. Very often, this diaphragmatic area prevents good neck rotation.

3. Positioning and Release:

- Position yourself and let the person fall back towards you.
- Ask them to relax completely. Support the person.
- Maintain a point of support with your thumb.

4. Descent and Tissue Listening:

- Start to descend gently, deeply. Feel for the tissues.
- Listen to the tissues. Observe the person's breathing. On exhalation, descend and meet a slight resistance, a certain density.

5. Stabilisation and Progressive Mobilisation:

- Position yourself gently, taking a good posterior support point, straight, to be stable.
- Check that the person does not feel any pain.
- Descend further, leaving a small margin due to clothing.
- Ask the person to sit up very gently. You can feel the "hop" come up with you.
- Repeat the movement: gentle ascent, then release. Gain ground.

- Sometimes, feel a "brake" and search a little further, ascending and descending again.

6. **Posterior Hold and Sensation:**

- Then take the posterior part between your two hands.
- Ask the person to relax completely. Check for the absence of excessive pain.
- A sensation may be felt, then "opens at the gallbladder level".

7. **Post-Technique Verification:**

- Seek posterior support and return.
- Ask the person to turn their head to the left again to check mobility.

TECHNIQUE 2: RELEASE OF THE LESSER SAC AND STOMACH

This technique focuses on the lesser sac and oesophageal insertions. Do not forget the importance of the **ribs** in relation to the diaphragm, where there is a significant concentration.

PRACTICAL PROTOCOL

1. **Location:** Place yourself where the "hole" is made.
2. **Hand Positioning:** Place your hand flat towards the junction, at the level of the 7th, 8th, 9th ribs.
3. **Sliding and Release:** Slide your hand underneath and lay it flat. Then, come with the other hand.
4. **Leverage:** Release and create a small lever. This area is an important crossroads of tensions.
5. **Effects:**
 - This will release the entire stomach-cardia junction.
 - This will relax the muscular area of Treitz and associated structures.
 - This acts on this vital relationship.

GLOBAL SENSATION AND RETROGASTRIC POUCH

- Focus on a more global sensation. Touch the whole body.
- The **retrogastric pouch** is very important to mobilise. Ulcers are often posterior, and this pouch needs mobility for good peristalsis and function. It must be free.
- If it is not well free, it is not released in relation to the diaphragm.

ESSENTIAL ANATOMICAL LINKS

- The oesophagus and stomach are the first organs in relation to the base of the skull.

- They have a relationship with the **posterior mesogastrium**, the horn, and the left main bronchi. You can feel the warmth rise, then descend.

BIODYNAMIC APPROACH AND EMBRYONIC DEVELOPMENT

I am now exploring another level: the strength of **embryonic development**, support, power. I observe the respiration and perceive a fascial work that aims to create more space between the diaphragm and the posterior pulmonary area.

THE PERITONEAL PLANE

- On the peritoneal plane, the **left colic flexure** is the first fixed point of the global development of the peritoneum. It is the first point of support.
- Focus on this point of support, then on the point of mobility.
- The most mobile point is the **caecum**, the last to be put in place. Assess how it is.
- Between your two hands, you have the entire axis of rotation of the colon frame. The diaphragm works on these structures.
- In this space, you have all the ligaments, such as the **falciform ligament**.

THERAPEUTIC CHOICE

The practitioner can choose what to work on, but often the patient's body guides this choice.

- For example, a tension on the falciform ligament, or on its leaflet.
- Sometimes, there is the expression of a "gentle anger", as if there was a lack of posterior support, or a need for release here.
- This work is done in synchronicity with the tentorium cerebelli, the falx cerebri, at the level of the **crista galli**. We will work on the sinus.

EXPRESSION OF ANGER IN BIODYNAMICS

Anger can be expressed as an inability at a given moment to express oneself. This can date back a long time, linked to what could not be said. In biodynamics, I grasp the protocol by placing myself on the globality. I leave the thoracic respiration and enter the space, leaving it open. The body then guides me to points of support. "Flashes" may appear, indicating tension.

THE KEY TO THE BIODYNAMIC APPROACH (INITIATORY)

- Dive into the water (metaphorically).
- Listen to the movement.
- Feel the movement.
- Seek the **balance** (the point of equilibrium).
- Once you have found the balance point, listen to the silence behind it.

When the cells "respond" and open to reception, it is a cellular **"ignition"**.

PRECAUTIONS AND HAND POSITIONING

When approaching the diaphragm, it is about giving space to the diaphragm.

MISTAKES TO AVOID

- **Pressing on the head:** You must have an equitable sensation between your hands. A major mistake is to press on the person's head, which is very uncomfortable in the long run.
- **Poor posture:** Have good support on your elbows.

POSITIONING TIPS

Imagine you are eating garlic and taking up the right space. It is crucial to find the right space.

MEDITATIVE GUIDANCE AND THERAPIST'S STATE

In this practice, I will guide you into a **meditative** state.

- Position yourself just in the right position.
- As a receiver, try to define your feeling of the position.

SENSATION OF TOUCH

The touch is very light, with a lot of warmth. It is global; one has the impression that it extends to the feet. At the same time, there is this sensation that I am grasping the **"micro-crystalline field"**: all these cells, this small cellular network with their receptors. It is an enormous field.

THE GLYCOCALYX FIELD

The largest concentration of information in a **glycocalyx** field is found at the level of the **second duodenum** – the most informed part of the intestine. This field is present everywhere on the body, like a global field that encompasses everything. This allows for deep contact.